

TX Pediatric Therapy Market Fragmentation — County Opportunity Screen

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Question: Which Texas counties are underserved and fragmented enough to be good acquisition or de-novo targets for pediatric speech, OT, and PT services?

Methodology

Providers: NPPES NPI Registry, org-level (NPI-2) records for Speech-Language Pathologist, Occupational Therapist, and Physical Therapist taxonomies in TX, name-matched for pediatric signal (org name or DBA containing "pediatric," "kids," "child," or "peds"). **Population:** Census ACS 5-Year 2023, under-18 population by county. **Geography:** Census ZCTA-to-County Relationship File maps each provider's ZIP to a county (no live geocoding).

Scoring: a 2x2 quadrant on two independent axes — no blended index with invented weights.

- **Density** = pediatric providers per 10,000 under-18 residents. Low = underserved.
- **Fragmentation** = % of pediatric provider locations belonging to an org (by DBA, or legal name) that appears at only one address statewide. High = fragmented, i.e. small independent operators rather than an existing multi-location competitor.

The target quadrant is **low density + high fragmentation**: underserved markets where existing providers are small enough to be realistic acquisition or partnership targets, not scaled competitors.

Top 5 Recommended Counties

Filtered to counties with ≥2 captured providers (a 1-provider county is trivially "100% single-location" and not a meaningful fragmentation read), ranked by density ascending:

County	Under-18 Pop.	Providers	Density /10k	Single-Location %
Tarrant (Fort Worth)	548,477	3	0.05	100%
Bexar (San Antonio)	508,025	4	0.08	100%
Denton (DFW suburb)	224,897	2	0.09	100%
Harris (Houston)	1,243,425	11	0.09	100%
Travis (Austin)	267,667	4	0.15	100%

These are Texas's largest metros, not overlooked small towns — and that's the point. Standard healthcare-services roll-up logic: large addressable child population, thin per-capita coverage even among the providers we can identify, and the operators we do find are single-location. That's a consolidation platform waiting to happen, not a market to avoid because it's "already saturated." Harris and Tarrant stand out — largest population base with lowest density in the set.

Data Limitations (read before acting on this)

1. **pediatric_signal is a name-matching heuristic**, not a clinical classification. It will miss practices with non-obvious names, and can be noisy in the other direction — one captured record was a pediatric *gastroenterology* practice pulled in via a tangential speech-therapy staff taxonomy, not a therapy clinic itself.
2. **The provider universe is a name-matched subset, not a census.** A first-pass pull of every TX org carrying an SLP/OT/PT taxonomy topped 120,000 records with no sign of terminating — dominated by home health agencies, hospices, and hospital systems listing therapy as one of many services, not pediatric clinics. That set is intractable and irrelevant here, so providers were instead found by searching organization-name keywords ("pediatric," "kids," "child," "peds") crossed with each taxonomy — the same heuristic, pushed into the query. **Effect: absolute provider counts and density are conservative undercounts everywhere.** Read rankings as directional/relative, not an exact census.
3. **Fragmentation likely skews toward "single-location."** A multi-location group whose branches don't all carry a pediatric-signal name will look more independent than it is. This affects every county the same direction, so relative comparisons hold up better than any one county's absolute number.
4. **County assignment is ZIP-to-county, not exact address geocoding** — standard practice at this level of aggregation, but a provider near a county line could be attributed to the wrong side in rare cases.

Bottom line: this is a defensible first-pass screen built on free, public data in a few days, not a substitute for a real census. A production engagement should license complete rosters from the TX speech-language pathology, OT, and PT licensing boards — which list every licensed practice location — to get reliable absolute counts and fragmentation figures before committing capital to a specific county.